
The Four Numbers That Predict Your Heart Attack

Before Your Cholesterol Does

The tests your blood panel is missing.
Built for South Asian men.



A LETTER FROM

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I am not writing this from a distance.

I lost a close friend at 34. He looked fine. Ate well, stayed active. His cholesterol came back normal three months before he died. His doctor saw no reason to worry. Neither did he.

He was not unlucky. He was under-tested.

The standard cholesterol test measures the weight of cargo moving through your blood vessels. What it does not measure is the number of particles carrying that cargo. Think of it like a highway — you can have five heavy trucks or a thousand small cars, same total weight, very different crash risk. Each of those small cars can embed itself into the wall of a blood vessel, build up slowly over years, and eventually block it entirely. When the vessel feeding your heart gets blocked, that is a heart attack.

South Asian men run more of these cars than almost any other population on earth. Even when the cargo weight — the cholesterol number — looks perfectly normal.

There are four numbers that see this clearly. **The test exists. Nobody ordered it for you.**

INFOGRAPHIC — REPLACE WITH NANO BANANA IMAGE

Two side-by-side panels. Left: LDL — few large trucks on a clear road. Label "Measures cargo weight."
Right: ApoB — many small cars crowded on the same road, one car embedding into the vessel wall. Label "Counts the cars. One car can cause a blockage."
Warm cream background. Red cars. Dark charcoal text. Flat editorial style.

Your last blood test probably missed all of them.

Screenshot this page. Bring it to your next appointment.

ApoB

< 70 mg/dL

The cars. Not the cargo.

Each ApoB particle can embed into your artery wall, build up, and block blood flow to your heart. That blockage is a heart attack. Standard cholesterol does not count these particles. ApoB does.

There is no safe number. The lower your ApoB, the lower your risk — full stop. The reference range on your blood test is not your target. Ask what your optimal number is, not just whether you are in range.

If high: Effective and safe treatments exist. Ask your doctor what is right for you.

Ask for: "Can you add ApoB to my blood panel?"

Lp(a)

< 75 nmol/L

The card you were born with.

Lp(a) is genetic. You inherited it from your parents. Diet, exercise, and most medications will not change it. If it is elevated, it makes every other risk factor in this guide hit significantly harder — more particles embed, more damage accumulates, faster.

You only need to test this once in your life. But you need to know the number.

If high: Treatments are currently in clinical trials — ask your doctor about the latest options. Managing your other three numbers aggressively becomes critical.

Ask for: "Can I get my Lp(a) tested? It is a once-in-a-lifetime test."

hs-CRP

< 0.6 mg/L

How inflamed your arteries are right now.

Inflammation makes artery damage worse. It makes the vessel walls more porous, more likely to trap particles, and more likely to rupture. High hs-CRP means this process is already active.

The biggest driver in South Asian men is visceral fat — the fat packed deep around your organs, around your liver and stomach. This is not the fat you can see or pinch. You can carry dangerous amounts of it at a completely normal body weight.

If high: Visceral fat is the priority target. Safe and effective fat loss treatments now exist alongside diet and exercise — ask your doctor.

Ask for: "Can you add high-sensitivity CRP to my panel?"

HOMA-IR

< 2.0

How hard your pancreas is working.

Insulin is a key. Your cells are like tiny rooms, each with a door. When you eat, insulin turns the key, the door opens, and sugar enters your cells to become energy.

Now imagine the locks get rusty. Visceral fat — packed around your liver and pancreas — does exactly this. Your body cannot tell the difference between a rusty lock and a missing key. So it makes more keys. More and more insulin, just to force the doors open. Blood sugar looks normal. But only because your pancreas is working five times harder than it should.

HOMA-IR catches this years before blood sugar moves. For South Asian men, the locks rust earlier — even at normal weight, even with a normal BMI.

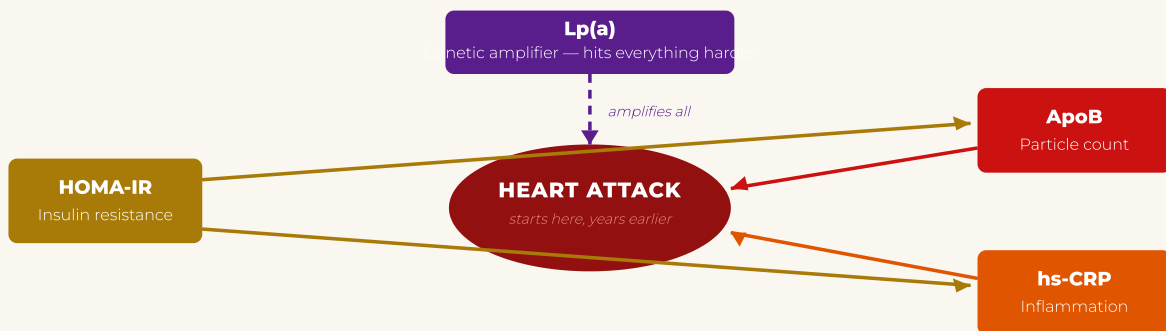
If high: Visceral fat is the root cause. A fibre and protein rich diet helps — specific targets vary. Safe and effective fat loss treatments now exist alongside exercise — ask your doctor.

Ask for: "Can you add fasting insulin to my panel so I can calculate HOMA-IR?"

These are not four separate problems. They are one system.

Insulin resistance drives visceral fat. Visceral fat produces more ApoB particles and raises inflammation. More particles means more material embedding into artery walls. Higher inflammation means those walls are more vulnerable. And if you were born with elevated Lp(a), every part of this process hits harder and earlier.

This is why a normal cholesterol result can coexist with a heart attack ten years later. The system was already running against you. These four numbers see it before the damage is done.



WHAT TO SAY TO YOUR DOCTOR — PRINT THIS PAGE

"I would like to add four specific tests to my next blood panel. I have been reading about heart attack risk in South Asian men and I want to establish my baseline on each of them."

- **ApoB** — fasting blood draw, report in mg/dL. Target: below 70.
- **Lp(a)** — once-in-a-lifetime test. Ask them to confirm the unit is nmol/L.
- **hs-CRP** — specify high-sensitivity CRP, not standard CRP. Target: below 0.6.
- **Fasting insulin** — combined with fasting glucose, this gives your HOMA-IR score.

If your doctor will not order one or more of these: all four are available privately at any accredited diagnostic lab without a referral. ApoB and hs-CRP typically cost less than a meal out.

You now know more than most South Asian men ever will.

SEND THIS TO SOMEONE YOU ARE WORRIED ABOUT

Your brother. Your cousin. The friend who keeps saying his cholesterol is fine. He almost certainly has not heard of ApoB. He has not had his Lp(a) tested. This guide takes ten minutes to read. It may be the most useful ten minutes he spends on his health this year.

Forward the email. Share the link. Screenshot page three and send it. Whatever gets it to him.

FOLLOW BROWN HEART FOR THE WEEKLY BREAKDOWN

Every week I break down exactly what is happening inside South Asian men's bodies — and what to do about it. No generic Western advice. No cholesterol myths.

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A QUESTION ABOUT YOUR RESULTS?

Reply to the email that brought you this guide. I read every one.

Brown Heart is an AI-assisted health education brand. Vaidya Aran is an AI-generated physician persona. Clinical content is authored and reviewed by a DHA-licensed physician with postgraduate training in biomedical informatics. This guide is for information only and is not a substitute for personal medical advice. Always discuss results and treatment decisions with your own doctor. Key references: Sniderman et al., JAMA Cardiology 2019; Tsimikas, JACC 2017; Ridker PM, Circulation Research 2016; Matthews et al., Diabetologia 1985; Enas et al., Indian Heart Journal 1996. · thebrownheart.health

